ResearchVoice AI | Business Brief

PHASE 1 • AI for Healthcare • Concept-stage business case • DRAFT / Requires team approval

Problem and estimated impact

Clinical researchers and physician-investigators repeatedly transfer study observations from interviews or notes into Excel, forms, and case-report records. This interrupts collection and creates checking and correction work, especially during follow-up visits. Today the workaround is manual typing and rechecking. ResearchVoice AI proposes a faster, researcher-controlled voice/text-to-structured-record workflow.

Illustrative planning estimate (NOT a measured baseline): 8 minutes per visit x 100 visits/month = 800 minutes (13 h 20 min) of monthly entry time. A target of 5 minutes per visit would reduce this modeled workload by 300 minutes/month. Both assumptions require a timed comparison before any benefit is claimed.

Objectives and audience

Proposed measurable objectives: (1) reduce average preparation time to <=5 min/visit, subject to validating the 8-min baseline; (2) achieve >=95% exact field-level agreement against synthetic ground truth before researcher corrections; (3) ensure 100% of delivered XLSX visits have explicit researcher confirmation.

Main user: clinical researcher / physician-investigator collecting repeated observational variables and follow-up records. Beneficiaries: research coordinators, data managers, and study teams. Potential buyer: a principal investigator, research unit, or university/hospital research administration (specific budget holder NOT confirmed).

Persona: a hospital-based researcher uses paper/CRFs, EHR notes and spreadsheets; regularly repeats the same variable entry; needs speed without accepting silent AI guesses.

Success metrics — proposed KPI targets

Lens	KPI	Baseline	Target / measure
Cost	Mean minutes to approved XLSX	8 min planning estimate only	<=5 min; time 10 paired synthetic cases
Outcomes	Exact extraction field agreement	Not measured	>=95%; compare to synthetic gold labels
Outcomes	Field correction rate	Not measured	<=15%; corrected/eligible fields
Outcomes	Approved export success	Not measured	>=95%; delivered/approved attempts
Workforce	Perceived entry effort	Not measured	>=4/5 ease score; short tester survey

Scope, safety and validation

IN: researcher-approved study schema; synthetic case codes/visits; Telegram text/voice; Groq transcription; Gemini schema-bound extraction with evidence; correction/approval; XLSX; basic QA monitoring. OUT: real patient records, diagnosis, prescribing, EHR write-back, production-grade data storage, autonomous decisions, or clinical benefit claims.

Synthetic-only validation targets from earlier pitch (NOT results): 20 researcher interviews, >=12 interested testers, 10 expected pilot users, and two interested research teams. No sales or quantified ROI claimed.

Evidence: working synthetic Telegram/Excel path, judge scores, LangSmith QA checkpoint and /monitor demonstrated. Still pending: measured KPI actuals, buyer confirmation, team/instructor sign-off.